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Assessment of a Clinical Pharmacist-Driven Appeal Process in a Dermatology Practice



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Background

- Insurance-driven prior authorizations (PA) have become a routine requirement to accessing prescription medications.
- The goal is to review medication appropriateness, however, these processes contribute to treatment delay and can lead to frustration from patients, providers and pharmacies.
- Specialty medications are typically high cost, resulting in frequent PA requirements.
- Surveys have shown that over 60% of specialty patients had some difficulty obtaining their first dose of therapy and almost 1 in 10 patients waited 8 weeks or more to receive their first dose.¹
- Current literature has focused on the benefit of adding an embedded clinic pharmacist to help with the appeal process in disease states such as hepatitis C and cardiovascular disease.^{2,3}
- The field of dermatology presents a unique opportunity for pharmacists to be involved in the appeal process due to the increase in prescribed specialty medications for conditions like psoriasis, alopecia, atopic dermatitis, hidradenitis suppurativa and vitiligo⁴.

Objective

Evaluate the impact of a clinic-embedded pharmacist on the rate of specialty medication appeal submission in a dermatology practice

Methods

- Retrospective, single-center review approved by the Wake Forest Baptist Health (WFBH) Institutional Review Board of adult patients with at least 1 specialty prescription medication that required a PA at the WFBH Dermatology Clinic between August 1st, 2018 and May 31st, 2019 and August 1st, 2019 to May 31st, 2020.
- Primary Endpoint: Change in the Rate of Appeals Submitted
- Secondary Endpoints: Change in the Rate of Appeal Approval, Number of Appeals Submitted, Time to Appeal Submission

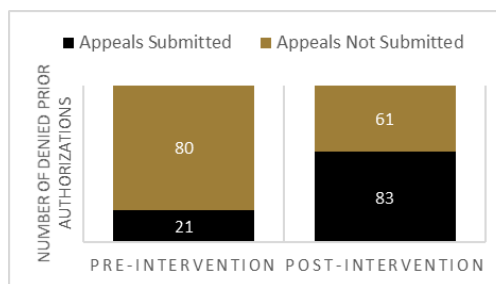
Dermatology Appeal Process Workflow

- Historically, nurses at the WFBH Dermatology Clinic managed appeals for denied prior authorizations concurrently with their primary clinical duties.
- Pharmacy presence began in 2018 with the addition of a medication access specialist (MAS). This role is fulfilled by a certified pharmacy technician with trained expertise in medication access.
- In 2019, a pharmacist was embedded into the clinic to support patient care including managing the appeal process.
- In the current process, all specialty medications requiring a PA are routed first through the MAS for completion and submission of the PA on behalf of the provider team.
- If the PA is denied, the appeal is completed and submitted by the pharmacist either by phone call or letter.



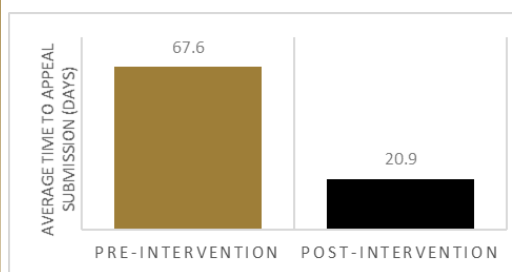
Results

Rate of Appeal Submission



The rate of appeal submission increased by 36.8% with the addition of a clinic-embedded pharmacist in the dermatology practice (20.8% vs. 57.6%, $p<0.001$).

Time to Appeal Submission



- A reduction of 46.7 days was seen in the average time to appeal submission (67.6 days vs. 20.9 days, $p<0.001$).
- The rate of appeal approval showed an increase of 17.4% with the addition of a clinic-embedded pharmacist (47.6% vs. 65%, $p=0.05$).

Discussion

- Prevalent diagnosis codes that were observed in this review included psoriasis, atopic dermatitis, intrinsic (allergic) eczema and alopecia areata.
- The majority of appeals were completed for off-label use, formulary and step therapy requirements.
- Certain manufacturer assistance programs require 1 or 2 appeals to be completed and denied prior to providing financial assistance.
- The rate of appeal approval was higher post-intervention but was not significant which may be indicative of the inability to control the success of the appeal determination process.
- Limitations include the single-center site location, inconsistency with PA documentation pre-intervention and unequal levels of medication access education between nurses and pharmacists.

Conclusions

- The presence of a clinic-embedded pharmacist in a dermatology practice positively impacted the rate of appeal submission, the rate of appeal approval and time to appeal submission.
- Dermatology provides an optimal environment for the addition of clinical pharmacy services to assist with medication access, especially due to the increase in specialty medications for dermatologic conditions.

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